



Clinical Practice Guideline

These guidelines are intended to guide clinical decision making for clinical staff at North Metropolitan Health Service. They are not strict protocols and do not replace clinical judgement. These guidelines should never be relied on as a substitute for proper assessment of the particular circumstances of each case and the needs of each patient.

If staff decide, in exercising their clinical judgment, not to follow the guideline, it is recommended that the reasons are clearly documented in the patient's health care record.

Peripheral Intravenous Catheters (PIVC) Insertion, Maintenance and Removal

Scope

Site	Service/Department/Unit	Disciplines
SCGH and OPH	Organisation Wide	All Staff

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Introduction

All Peripheral Intravenous Catheters (PIVC) provide direct access to the patient's bloodstream and therefore pose a serious risk for infection from microorganisms introduced either at the time of insertion or while the cannula is in situ. PIVC related infections are associated with increased morbidity, mortality, hospital length of stay and healthcare costs^{1,2}. Infections associated with PIVCs are considered preventable adverse events.

Data from Healthcare Infection Surveillance WA (HISWA) consistently shows most healthcare-associated Staphylococcus aureus bloodstream infections (HA-SABSI) occur because of intravascular devices, with a large percentage attributable to PIVC.

Prevention of PIVC-related complications requires a combination of processes including strong clinical governance in relation to provision of training and education, avoiding unnecessary PIVC insertions, promotion of early PIVC removal, and support for infection prevention practices utilised during insertion and management of these devices.

Risk Statement

Non-compliance with this guideline may:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☐
Other:	☐		

Definitions

Peripheral intravenous catheter (PIVC)	A device that is designed to be inserted into, and remain within, a peripheral vein (excludes peripherally inserted central catheters). Peripheral veins are those in the arms, legs, hands and feet.
Healthcare Worker (HCW)	Individual who practises in a health profession (e.g. Medical Practitioners, Nurses, Midwives) and who is registered and able to provide medical and healthcare treatment to a patient.
Peripheral intravenous assessment score (PIVAS)	A validated tool for evaluating and documenting the status of PIVC sites.
Difficult Intravenous Access (DIVA)	Defined as a patient that has limited visible and palpable veins and or multiple unsuccessful attempts to insert a catheter. This can be acute due to sudden illness, or chronic resulting from complex medical intervention. Characteristics of DIVA include, but are not limited to: • patient characteristics e.g. overweight (BMI greater than 30) or underweight, extremes of age (history of prematurity and older adult), and gender (female) • a stated history or documented history of difficulty obtaining vascular access • a history of drug abuse.
Aseptic Technique	A technique during invasive clinical procedures that aims to prevent microorganisms on hands, surfaces and equipment being introduced to susceptible sites.

Aim

This guideline aligns with the [MP 0038/16 Insertion and Management of Peripheral Intravenous Cannulae in Healthcare Facilities Policy and Guideline](#). It provides guidance on the minimum practice standards required at SCGOPHCG for insertion, management, and removal of PIVCs to protect patients from PIVC associated adverse events.

General Safety Information

To reduce avoidable complications, it is essential that PIVC's are used judiciously.

- Phlebotomy alone is NOT an indication for PIVC insertion.
- Prior to any intravenous access, identify the safest and most appropriate route of administration for clinical need. Consider another route of administration, e.g., oral, intramuscular, subcutaneous, rectal, intra-osseous or intra-peritoneal routes, before using the intravenous (IV) route ⁽¹⁻⁴⁾
- If IV access is required, determine whether peripheral or central venous access is appropriate by considering:
 - Patients medical history, age, clinical and vascular condition⁽¹⁻⁴⁾
 - Expected duration of therapy. If prolonged IV therapy, (greater than 6 days) is likely to be required, consideration for a central catheter should be given⁽¹⁻⁴⁾.
 - Likelihood, repeated or prolonged administration of vesicants or irritants⁽¹⁻⁴⁾
 - Likelihood of first-time insertion success⁴.
- All PIVC must be inserted and maintained by, or under the direct supervision of, a healthcare worker (HCW) working within their scope of practice and experienced in PIVC insertion.
- Staff are encouraged to risk assess all PIVC insertions with consideration for maximising first insertion success. This includes:

- Early assessment to identify patients who may have Difficult Intravenous Access (DIVA)
- Escalation to more experienced clinicians where complexity of the insertion is outside of the clinician's expertise or where the inserting HCW has had 2 failed attempts to cannulate the same patient
- Standard precautions must be followed during the insertion, management, and removal of PIVCs. Standard precautions include the 5 moments for hand hygiene and principles of aseptic technique, appropriate use of PPE to prevent blood and body fluid exposure, safe handling and disposal of sharps, use of sharp safety engineered medical devices and correct waste management procedures.
- Assessment of the PIVC and ongoing requirements is to occur at each medical ward round and nursing handover, using DRIP criteria (refer to [Appendix 2: Peripheral Intravenous Assessment Score](#)).

1. Patient education and consent

Unless in an emergency, ensure that the need for IV access is discussed and understood by the patient before the PIVC is inserted. The IV-Wise patient discussion tool provides key discussion points for education about the PIVC to support patient shared decision making, informed consent and active participation in preventing PIVC related complications⁴. A minimum of verbal consent is required prior to PIVC insertion.

If the patient is discharged from hospital, explain what signs they should look out for after the PIVC is removed and who they should contact if signs of infection develop.

Offer the patient a consumer information leaflet "*Looking after your cannula*" (Australian Commission on Safety and Quality in Health Care),. This is available on the SCGOPHCG intranet.

2. Competency requirements for staff new to cannulation

Staff who have not previously cannulated should be supervised and deemed competent to perform this procedure.

Nurses must only cannulate the lower arm, inclusive of the antecubital fossa and dorsum of the hand. Nurses must not cannulate patient's feet.

Only nurses with experience in renal and oncology, are able to perform peripheral IV cannulation on these categories of patients.

Part One: Complete the theory

- Read and complete the PIVC Self Directed Learning Package on Share Point and the Intravascular Access Service Share Point page
- Complete the SCGH Aseptic Technique e-learning Package

Part Two: Complete the practical assessment

- Staff can be supervised and deemed competent by those already proficient in cannulation at SCGOPHCG. Two successful supervised insertions on a patient should be completed.
- Competency can only be maintained if the skill is practiced regularly. Once competent, it is up to the individual to maintain their practical skill and continuing education⁴.
- A certificate of completion can be obtained from the Nurse Led IV Access Service, once competency is achieved.

3. PIVC site selection

PIVC's are to be routinely sited in the distal areas of the upper limbs (e.g., forearms). Subsequent PIVC's are to be inserted where possible, proximal to the previous site. Select

the most appropriate vein for insertion of the PIVC with consideration of:

- Indication and expected duration of treatment.
- Size and condition of the patient's veins.
- Utilising the non-dominant forearm if practical.
- Patient positioning and PIVC accessibility during surgery and procedures.
- Use of the basilic or cephalic veins on the posterior (dorsal) forearm are the preferred site for cannulation.
- Rotate PIVC site and arm where possible for repeated cannulations ^(1-4,6).
- Excessive hair around the intended insertion site must be removed with surgical clippers (not shaved) to facilitate adequate dressing securement.

AVOID THE USE OF VEINS IN THE FOLLOWING SITES:

- Areas of flexion e.g. **antecubital fossa (ACF)**, or bony prominences due to significantly increased risk of bloodstream infections (BSIs), PIVC failure and discomfort for the patient. Except in the case of an emergency, if the only site for PIVC insertion appears to be the antecubital fossa, the patient should be escalated to a senior clinician or referred for DIVA assessment where possible.
- Veins of the lower extremities, due to risk of tissue damage, thrombophlebitis, ulceration, reduced access, patient comfort and mobility.
- Areas below previous cannulation, sites of bruising, erythema or painful on palpation, phlebitic areas or areas with loss of skin integrity^{2, 3}
- The anterior (ventral) forearm veins, especially the cephalic vein, in patients with chronic renal failure, as these may be required for fistula formation for dialysis
- For any patients with a fistula or shunt please refer to the [SCGOPHCG Vein Selection and Preservation in Patients with Chronic Kidney Disease Guideline](#) for PIVC placement options and recommendations.
- The arm on the same side as a previous axillary clearance, mastectomy, where there is active lymphoedema. Note: It is no longer a contraindication to have a PIVC inserted in the arm of the affected side of patients who have had removal of axillary nodes or clearance. However, caution should be exercised, and such patients referred to the IV Access Service for assessment, and optimal PIVC placement.⁶
- The arm on the side that has been affected by a cerebrovascular accident²
- A limb with a PICC or implanted venous access device²
- An infected limb e.g. with cellulitis, due to increased risk of infection.

Intravenous cannulation of the foot should be a last resort and not be used repeatedly. If this has been performed, the site must be closely monitored at least once each 8 hours for signs of complications (i.e., swelling, pain, oedema). A referral for definitive vascular access assessment and assistance should be made to the Nurse Led IV Access Service or Anaesthetic service as a priority, and the PIVC should **not** remain insitu beyond 24 hours.

4. PIVC device selection

- Clinicians should use the smallest gauge PIVC that will accommodate the anticipated clinical need (e.g. IV hydration, blood products – see [Appendix 1: Selection Guide](#)).
- PIVCs classified as safety-engineered medical devices (SEMDs) should be used. Exceptions may include when an alternative PIVC is required for specialist procedures.
- Any PIVC to be left *in situ* (except PIVC requiring short dwell time, < 6-12hrs, in the outpatient setting, in an emergency or procedural situations) should have an extension set attached e.g., J loop. The extension set maintains stability, reduces trauma and phlebitis and increases PIVC longevity. The extension set is to be changed when each PIVC is re-sited.

- For Radiological procedures, such as CT or MRI, a 20g PIVC is suitable, with preference for the right arm. (This does NOT need to be inserted into the ACF).
- For Coronary angiogram, the ideal size PIVC is an 18g.

5. Aseptic Technique

Aseptic technique is essential during insertion and maintenance of the PIVC. This includes:

- Attention to the 5 Moments for Hand Hygiene.
- The use of a dedicated IV trolley, available on all wards, to facilitate aseptic technique. Prepare the trolley prior to use by wiping the surface with a combined detergent/disinfectant wipe and allow to dry.
- Sterile gloves must be worn during PIVC insertion.
- A sterile PIVC starter pack or sterile dressing pack and associated sterile equipment should be used to insert all PIVC's. Prepare and manage the equipment for the procedure in an aseptic manner and protect key parts and key sites using a non-touch technique.
- Adequately prepare the skin. Liberally disinfect a large area of skin around the insertion site to ensure the dressing is applied to disinfected skin. Allow skin antiseptic to air dry to ensure adequate contact time. Do not wipe or blot skin to increase drying time.

6. Ultrasound Guided Insertion Procedure

6.1 Operator Requirements

If a PIVC is inserted using ultrasound, the insertion must be performed by, or under the direct supervision of a HCW experienced in the use of ultrasound.

6.2 Ultrasound Transducer (probe) classification

- 6.2.1 **Non-invasive ultrasound transducers**: Used in percutaneous procedures such as PIVC insertion.
- 6.2.2 **Invasive ultrasound transducers**: penetrate inside the body through areas of non-intact skin (e.g. burns sites, open wounds, skin ulcers).
- 6.2.3 Reclassification: A non-invasive transducer becomes invasive if *at any time*;
 - It directly contacts a puncture site without a sheath (including previous PIVC insertion attempt sites)
 - The sterile probe cover is damaged
 - It contacts ulcerated/non-intact skin without a sheath
 - There is visible blood or body fluid contamination on the transducer

6.3 Reprocessing Requirements and Procedure

- Non-invasive ultrasound transducers are considered non-critical medical devices, and MUST undergo, at a minimum, low-level disinfection (LLD) between procedures with a combined detergent / disinfectant wipe e.g., Clinell™
- Invasive ultrasound transducers require High Level Disinfection (HLD)
- Prior to use, both invasive and non-invasive ultrasound transducers require LLD, even if HLD has occurred.
- If at any point during the use of the probe it becomes an invasive medical device the probe MUST be sent for HLD as soon as possible after use.
- In hours, the ultrasound user is required to contact the IV Access Service to facilitate HLD. After hours, please take the machine to the shift coordinator on ward G53, G63, G73, C14, respectively who will take the machine off the floor and contact the IV Access Service in the morning for processing.
- All efforts should be made to keep transducers used for PIVC insertion in a non-invasive state to avoid the need for HLD.

6.4 Use of Sterile Transducer (probe) Covers

- The use of sterile probe covers is mandatory while performing PIVC insertion as part of sterile insertion technique.
- Sterile probe covers are also strongly advised when screening for veins in patients who have had recent previous PIVC insertion attempts or areas of non-intact skin, to avoid transducers being reclassified as invasive avoid the need for HLD
- Probe covers, sterile gel, and long cannulas are stocked (at minimum) at SCGH on G53, G63, G73, and C14 where the ward ultrasound machines are found, and at OPH are found in Theatre. Please inform ward shift coordinators if there is low stock.

7. Securing the device and dressing management

- Use a sterile, transparent, semi-permeable dressing to secure the PIVC, extension set, or needleless valve, to allow continuous observations of the site and protect the insertion site from contamination and dislodgement. Opaque tape should not be placed directly over the insertion site.
- Taking care not to contaminate the adhesive part of the dressing, secure the dressing where the cannula hub and the extension set connect and ensure the dressing is firmly adhered to the skin.
- Record the date and time of insertion and the signature of the inserter on the adhesive strip of the IV dressing.
- Ensure the PIVC is kept dry. Use a waterproof cover over the PIVC site when showering the patient. If dressing becomes wet, loose or contaminated clean and replace with a new dressing using an aseptic non-touch technique, as per re-dressing instructions. Document in the patients' medical records
- If a PIVC becomes dislodged or inadvertently partially withdrawn, it must be removed and a new PIVC inserted, as soon as practical.
- Ensure PIVC and all IV lines are secured to minimise the risk of drag and PIVC dislodgment.

8. PIVC assessment

At least daily, the medical and nursing teams are to assess the need for the PIVC against the DRIP criteria, with the view to remove the PIVC when clinically indicated or no longer required.

At least once per shift, or each time the PIVC is accessed:

- Monitor the insertion site and perform a peripheral intravenous assessment score (PIVAS) for the duration that the PIVC is insitu and for 48 hours post removal, or longer if clinically indicated.
- Inspect for:
 - signs of pain, exudate, swelling or redness at the insertion site, by visual inspection through the transparent dressing as well as gentle palpation through the dressing to assess the condition of the patient's veins, and whether they have become hardened or thrombosed²
 - signs of localised or systemic infection. If either are confirmed, report in an incident management system² (refer also to [Section 17 Removal of the PIVC if Infection Suspected](#))
 - for leakage of fluid from the insertion site, signs of occlusion, infiltration or extravasation²
 - that the PIVC remains appropriately dressed and secured.²
- All PIVCs are to be assessed for patency and for any signs of complication each time the device is accessed and determine if the patient is tolerating their PIVC.

9. PIVC documentation and monitoring requirements

The tool for documentation of PIVC insertion and ongoing management is the SCGOPHCG PIVC Insertion and Observation Record ([Appendix 3](#)) and the SCGOPHCG PIVC Insertion and Observation Record – DIVA ([Appendix 4](#)) for DIVA patients (for further details on DIVA documentation requirements please see section 15). Insertion details must be completed on these forms at time of insertion; including details below.

9.1 Documentation

The record of insertion is to be completed by the inserting HCW and is to include:

- Date and time of insertion.
- Anatomical site of insertion.
- Name and designation of HCW inserting the PIVC.
- Identification of whether PIVC insertion occurred during an emergency, and/or by an external HSP.
- Indications and decisions for PIVC insertion and a management plan, using DRIP criteria.

The IV insertion label is to be used as a visual prompt, noting the date and time of insertion and signature of the HCW inserting the PIVC. Place on the external transparent PIVC dressing (without obscuring the PIVC insertion site and ability to assess). (Refer to Image 1).

9.2 Monitoring

All clinical interventions for each PIVC site are to be recorded in the patient's medical record. At a minimum, maintenance documentation of the PIVC is to include:

- The peripheral intravenous assessment score (PIVAS) each shift or at least every 8 hours while the PIVC is insitu and for 48hrs following removal.
- PIVC removal date, time, and reason for removal.
- Any PIVC complications and associated clinical interventions.
- Clinical assessment and indications to support decisions relating to the assessment of DIVA and/or extended dwell time.

All clinical incidents are to be reported via CIMS and as per [NMHS Clinical Incident Management Policy](#)



Image 1: CHG dressing application for DIVA and HITH patients



Image 2: PIVC dressing application

10. PIVC blood collection

- Blood may be drawn from the PIVC directly after insertion, but **NOT** at other times. The exception is during an emergency when the patient has limited vascular access or is at increased risk of bleeding or receiving thrombolytic therapy²
- Blood cultures are not to be collected through the PIVC.

11. Needleless access ports

- Needleless access ports are to be placed and used on all lumens. Stopcocks are to be end-capped with a needleless access port when not in use.
- All PIVC access ports are to be disinfected by applying mechanical friction with a single-use 70% alcohol-impregnated swab and allowed to dry prior to accessing the system.
- Access ports are only to be accessed with sterile single use devices.
- When an access port is removed from a PIVC or extension set, it is to be discarded, and a new sterile access port attached.

12. Management of administration sets

- Administration sets are single use devices and if they are disconnected from the IV cannula for any reason e.g. intermittent medication dosing, the set is to be discarded and a new administration set connected using aseptic technique.
- Continuous use administration sets, including all tubing, connections, extension sets, and needleless valves are to be changed each 72 hours and/or when a PIVC is re-sited.
- Administration sets are to be changed more frequently if contamination or accidental disconnection occurs, or a blood reaction is suspected.
- When blood or blood products have been infused, change the administration set, including all IV tubing and connections immediately after completion, or every 12 hours, whichever comes first.
- Lipid or lipid-containing parenteral nutrition administration sets must be changed within 24 hours.
- Chemotherapeutic agent administration sets must be removed immediately after use.
- Administration sets are not to be disconnected for routine care e.g. showering, but may be disconnected for transient, controlled disconnections e.g. changing IV access or infusions in the operating theatre or medical imaging department.
- Label all administration sets attached to the PIVC with an intravenous line label in accordance with the National Standard for User-Applied Labelling of Injectable Medicines, Fluids and Lines⁴.

13. Flushing and PIVC

Where possible, PIVC's are to have a continuous flow of IV fluids through them.

Decontaminate the needleless connector with a single use 70% alcohol impregnated swab before accessing.

Using standard precautions including hand hygiene and aseptic technique, a PIVC is to be flushed with 5-10mls of sterile sodium chloride for injection with a 10ml luer lock syringe or commercially available pre-filled syringe and using a pulsatile motion (push-pause/ start stop):

- After the PIVC is inserted and prior to each use to confirm patency. If significant resistance is encountered, the patient complains of pain on administration or fluid appears to be entering the subcutaneous tissue, do not use the PIVC. Liaise with a medical officer - remove the PIVC and arrange a re-site if appropriate.
- After each injection/infusion to remove irritant material from the vein.
- Between multiple infusions or medications to prevent interactions and incompatibilities.
- Prior to and following blood drawing (Refer to Section 7. PIVC Blood Collection).
- At least every 8 hours if the PIVC is not in use. Note: strong consideration should be given for removal of the PIVC if it has not been accessed for 12 hours.

Note: Disconnection of the flush syringe can allow reflux of blood into the tip of the

catheter to displace the space occupied by the syringe. To prevent this source of occlusion, clamp the extension set or withdraw the syringe while administering the last 0.5ml of flush (positive pressure technique).

14. Duration of use and re-siting PIVCs

PIVC's are not to remain in situ longer than 72 hours barring exceptional circumstances (e.g., Difficult Intravenous Access (DIVA) – refer to [Section 15. Extended Dwell Time](#)).

The need for the PIVC is to be assessed against the DRIP criteria each shift and removed as soon as it is no longer required. If the PIVC is to be maintained, the reason is to be documented in the medical record. If ongoing access is required past 72 hours, planned re-siting of the PIVC is to occur, during routine business hours wherever possible.

- If a PIVC is inserted into the ACF, or other suboptimal anatomical site (see section 3), early referral should be considered to IV access, anaesthetics, or an USS certified clinician for DIVA assessment to ensure timely re-siting to a more optimal site or alternative access if a PIVC is still required.
- All PIVC inserted by ambulance or other emergency services (e.g. RFDS) are to be removed as soon as the patient's condition has stabilized or within 24 hours of insertion and only replaced if clinically indicated.⁴
- Remove PIVC's that may have been inserted without adherence to aseptic technique e.g., during resuscitation, as soon as practical and within 24 hours of insertion.
- Any patient with a PIVC following inter-hospital transfer is to have their PIVC assessed for ongoing clinical need (DRIP criteria), time in situ and PIVAS score. If the assessment confirms the PIVC is clinically required and meets guideline standards, it may remain in place for up to 72 hours (or longer for patients identified as DIVA, in line with usual guideline process). If unable to determine adequate information, the PIVC should be removed and re-sited if there is ongoing clinical need.
- If the PIVAS is 1, remove the PIVC, arrange medical review, and re-site PIVC if there is ongoing clinical need, as per DRIP criteria.
- If the PIVAS is 2 or greater, remove the PIVC, arrange medical review, send the PIVC tip and a swab of the PIVC site for MC&S, collect two sets of blood cultures and initiate a CIMS.
- If a patient with a PIVC in place develops fever $\geq 38^{\circ}\text{C}$ or signs of sepsis, a medical review should be performed; the PIVC should be removed if it is a potential source of fever/sepsis. (Refer to [Section 17. Removal of the PIVC if Infection suspected](#)).
- If prolonged IV therapy is likely to be required, e.g., more than 6 days, consider referral for insertion of a central catheter, either peripherally or centrally inserted, or a long peripheral venous catheter (also referred to as a 'midline' catheter), rather than multiple replacement of PIVCs.
- If extravasation occurs special precautions are required prior to removal of the PIVC. Refer to local area guidelines.
- Routine culturing of PIVC tips is not recommended unless infection is suspected.

Continue PIVAS post PIVC removal for 48hrs and monitor the insertion site. If there are any concerns after 48hrs, escalate to the medical team and commence a management plan. If phlebitis is present and the patient is being discharged, refer to GP for monitoring and review and include this in the discharge summary.

15. Extended dwell time beyond 72 hours

The exception to the 72hr rule for PIVC re-siting is for patients with Difficult Intravenous Access (DIVA) or patients with a short, defined duration of IV therapy remaining under the conditions outlined below.

15.1 Difficult Intravenous Access (DIVA)

DIVA is defined as a patient that has limited visible and palpable veins and or multiple unsuccessful attempts to insert a catheter. This can be acute due to sudden illness, or chronic. Characteristics of DIVA include, but are not limited to:

- Diseases/conditions that affect the integrity of the vessel structure, such as diabetes
- A stated history or documented history of difficulty obtaining vascular access
- Presence of scars or tattoos
- Extremes of age (history of prematurity and older adult)
- Obesity or malnourishment
- Dehydration
- History of treatment with anticoagulants or corticosteroids
- A history of intravenous drug use⁴.
- Patients receiving Dialysis
- History of chemotherapy treatment

15.1.1 DIVA Procedure

- Multiple attempts at PIVC insertion increases the risk of blood stream infection and compromises vessel health⁴. If a patient has or is suspected to have DIVA, PIVC insertion should be immediately escalated to the most experienced local clinician and consideration given for the use of alternative intravascular access or the use of advanced insertion technique, such as the use of ultrasound guidance. Except in the case of an emergency, if the only site for PIVC insertion appears to be the antecubital fossa, the patient should be escalated to a senior clinician or referred for a DIVA assessment.
- When the most senior locally available clinician has been unable to successfully insert a PIVC within two attempts, the patient should be referred to another appropriate service. The Nurse Led IV Access Service offers a DIVA PIVC insertion service for SCGH patients. The Department of Anaesthesia does not offer an ad-hoc PIVC insertion service. However, the Duty Anaesthetist / CAT team can be consulted for advice on insertion of clinically essential PIVC's on suspected DIVA patients.

15.1.2 DIVA Confirmation and Documentation

DIVA status must be confirmed by the IV Access Service, an anaesthetist, or a clinician certified in USS guided cannulation to be considered a documented DIVA for the purposes of extended dwelling. DIVA can also be confirmed by ICU or ED consultants while under acute care. DIVA Assessment must be documented in the patient's medical record (DMR). Documentation may be recorded by a delegate of the assessor and must include the assessors name and designation. A referral must be made to the IV access service at time of DIVA determination (SCGH only).

If a patient's DIVA status is considered chronic, the IV access service can apply a medical alert to webPAS.

For DIVA patients, the SCGOPHCG PIVC Insertion and Observation Record – DIVA ([Appendix 4](#)) should be used and DIVA assessment section completed in full.

15.1.3 DIVA Extended Dwell Time

If a patient has a documented DIVA status, a PIVC may remain in place for up to 120 hours (5 days) if;

- PIVC remains clinically indicated per the DRIP criteria
- PIVAS remains 0,
- A referral is made to IV Access Service at time of PIVC insertion, with a daily review by the service after 72 hours (SCGH only)

The PIVC must be removed at 120 hours (5 days).

In the absence of documented DIVA, PIVCs must be removed at 72 hours from insertion and re-sited if clinically indicated. Planned re-siting of the PIVC is to occur, during routine business hours wherever possible.

After hours exception:

An exception to the above applies when DIVA is noted and the IV Access Team, an anaesthetist, or USS certified clinician are unavailable for assessment (e.g., during service overload or after hours) to document confirmation. In these circumstances, the PIVC may remain in situ for up to an additional 24 hours (96 hours post insertion) while awaiting confirmation, provided that:

- The PIVAS is 0,
- A referral is placed with the IV Access Service at the time the decision is made, and
- The rationale for DIVA and the ongoing need for the cannula (per DRIP criteria) are clearly documented in the patient's medical record by the clinician seeking documented DIVA status.

15.1.4 Osborne Park Hospital (OPH) DIVA Escalation Pathway

Management of PIVC insertion and DIVA at OPH is aligned with the hospital's service profile and the level of clinical support available.

In-hours

- For patients with suspected DIVA (i.e., patients meeting DIVA criteria), escalate PIVC insertion to the most experienced local clinician
- There are anaesthetists on site during business hours who provide operative services. They may be able to assist with DIVA but are not routinely available for PIVC insertion.
- The IV Access Service (SCGH) may be consulted in non-emergent situations when local resources have been exhausted or when advanced access (e.g., PICC) is anticipated.
- Every effort should be made to clarify the ongoing IV therapy plan prior to transfer to OPH, to allow appropriate vascular access to be arranged ahead of time.
- It is essential that the PIVC and PIVC management plan are assessed and clarified during routine business hours, allowing timely escalation for re-siting while more resources are available.

After-hours

- PIVC insertion is usually performed by the after-hours medical team (Registrar and RMOs), supported by nursing staff with IV access competency.
- The on-call anaesthetic consultant provides cover for obstetric and operative services and is not routinely available for PIVC insertion.
- PIVCs should remain in situ overnight if there is difficulty with re-siting, the PIVC is required per the DRIP criteria, and the PIVAS is 0.

Escalation / Transfer

- If vascular access cannot be established and time-critical therapy is required, escalation should follow the usual Recognising and Responding to Acute Deterioration pathways and the patient should be considered for transfer to SCGH for definitive vascular access as appropriate
- If IV therapy is not time-critical, cannulation may be deferred to business hours when additional supports or alternative access options become available.

15.2 Limited Extended Dwell Time in patients without DIVA

For patients without documented or suspected DIVA, the PIVC may be retained for up to 96 hours from insertion at the clinician's discretion when a short, defined duration of clinically necessary IV therapy remains, and it is in the patient's best interest to avoid

additional cannulation attempts. Examples include where less than 24 hours of IV antibiotics remain, or an iron transfusion is required prior to discharge. The clinical rationale for limited extension of PIVC use must be clearly documented in the patient's medical record and PIVAS must be 0. If this documentation is not present or insufficient, or PIVAS is above 0, the cannula must be removed. If the PIVC is unexpectedly required post 96 hours this period will not be extended and the cannula must be removed and re-sited at 96 hours post insertion per the usual 72-hour protocol.

16. PIVC in the home setting

If a patient is to be discharged home with a PIVC insitu (e.g., via HITH/Homelink) it must be deemed appropriate by the treating medical team. The patient must be medically stable, have a PIVAS score of zero and be provided with written instructions on how to manage their PIVC at home. Suitable criteria for home management of a PIVC includes:

- The patient must not be a documented DIVA, and the PIVC must be reviewed daily for ongoing clinical need.
- The PIVC should not remain insitu beyond 72 hours.
- The PIVC must not be inserted into the ACF.

Patients with a documented history of intravenous drug use, confusion, cognitive impairment, or history of non-attendance or discharge against medical advice, should not be discharged home alone with a PIVC insitu.

The use of a CHG antimicrobial impregnated dressing is preferred for securement of the PIVC.

The use of a CuroresTM disinfectant cap on the end of the j-loop is recommended.

17. Removing the PIVC if infection suspected

- If infection is suspected, inform the treating medical officer.
- Two sets of blood cultures are to be collected. Blood culture samples are to be drawn from another peripheral vein. Blood must **not** be drawn from the existing PIVC.
- Follow aseptic non-touch technique during sampling.
- Any PIVC site discharge should be swabbed and sent for culture.
- On removal of the PIVC, send the catheter tip for culture in a sterile screw top container (blood cultures must accompany the tip).
- Document all actions in the patient's medical record.
- Document in CIMS.
- Open disclosure should be performed for all suspected or actual catheter related infections, as per the [NMHS Open Disclosure Policy](#)

18. PIVC Procedural guidelines

18.1 Insertion

Requirements

- Personal Protective Equipment (PPE) as for Standard Precautions
- IV trolley / stainless steel procedure trolley
- PIVC Insertion Pack including the following OR the following individual items
 - Dressing pack
 - 2% Chlorhexidine in 70% alcohol swab stick or solution
 - Sterile occlusive transparent dressing (or CHG antimicrobial dressing used by DIVA service for patients suitable for prolonged dwell time or patients under the care of HITH going home with PIVC insitu)
 - Tourniquet (single patient use or hospital approved reusable device able to be cleaned and disinfected prior to use e.g., DaisygripTM)

- Extension set (e.g. J loop) and/or IV bung
- Waterproof/absorbent sheet
- Sterile covered pre-filled 10ml normal saline syringe OR Normal saline 0.9% 10 mL ampoule, 10mL syringe and drawing up needle
- Sterile gloves
- Peripheral cannula of appropriate size (one attempt only per cannula)
- Waste disposal bag
- Sharps container
- Surgical clipper (optional)
- Sterile ultrasound probe cover with sterile gel (if using ultrasound)

For local anaesthetic (if required)

- Topical local anaesthetic, as prescribed e.g. EMLA™ cream (to be applied locally one hour **prior** to procedure, cover with occlusive dressing). This can be initiated by a nurse. OR
- Lignocaine 1% (50 mg in 5 mL vial) (this must be prescribed by a medical officer on the medication chart)
 - 2 mL syringe
 - 26G or 30G needle
 - Drawing up needle

Note for patients with a chlorhexidine allergy / sensitivity, use povidone iodine 10% in ethyl alcohol. If alcohol is contraindicated, use aqueous 10% povidone-iodine solution.

Procedure

Ensure a PIVC is clinically indicated.

Note: Palpation of the PIVC insertion site should not be performed after the application of antiseptic, unless sterile gloves are used. If you need to palpate the planned insertion site after skin antisepsis to confirm anatomy, repeat the application of antiseptic².

The following workflow is suggested to support the principles of aseptic technique:

1. Confirm the patient's identity, explain procedure to the patient and obtain verbal consent
2. Check for patient allergies (in particular allergies to tapes, dressings and skin preparation)
3. Perform hand hygiene
4. Examine the arm for suitable veins and choose a site away from the joints or signs of inflammation
5. Where possible, ask patient to clean hand and arm with soap and water if skin is visibly soiled
6. Choose the site and clip excessive hair if required
7. Perform hand hygiene
8. Clean trolley with a combined detergent/disinfectant wipe. Allow to dry
9. Gather equipment including local anaesthetic if required
10. Perform hand hygiene
11. Prepare sterile field, open PIVC or dressing pack and add items and set out equipment using aseptic technique
12. Remove local anaesthetic cream (if used) with dry gauze
13. Place waterproof/absorbent sheet under patient's arm
14. Apply tourniquet
15. Perform hand hygiene and put on sterile gloves
16. Perform skin disinfection using 2% chlorhexidine in 70% alcohol solution or swab stick, except in the case of documented allergy. Liberally swab a large area of skin around the chosen site to ensure the site for the dressing is also disinfected. Allow antiseptic to air dry to ensure adequate contact time. Do not wipe or blot skin dry.

Note: skin disinfection should occur before donning sterile gloves if the swab stick is used.

17. Prime extension set or bung

If subcutaneous lignocaine is required using the drawing up needle, draw up 2 mL in a syringe, inject 0.1 mL subcutaneously. Aspirate to ensure you are in the skin not in the vein. If blood returns, remove needle. If no blood returns, proceed

If there is a need to re-palpate, only touch the cleaned area of skin

18. Apply skin traction and insert the PIVC into the prepared vein with bevel up at a 25° angle

- if unsuccessful, discard PIVC, remove sterile gloves, perform hand hygiene and prepare for insertion at a new site.

19. Observe cannula lumen for return of blood (flashback)

20. Advance PIVC into the vein

21. Release tourniquet and apply digital pressure above the insertion point

22. Attach the primed extension set or bung to the PIVC and ensure patency by flushing with 5-10 mL 0.9% normal saline. If using an extension set, ensure it is clamped post flush

23. Apply sterile transparent dressing over insertion site using a non-touch technique

24. The insertion site should remain visible for inspection. Place a signed, date label on dressing

25. Dispose of sharps and waste into the correct containers

26. Remove PPE including sterile gloves and perform hand hygiene

27. Clean trolley with combined detergent / disinfectant wipe

28. Perform hand hygiene

29. Document date and time of insertion, cannulation site, patient location when the PIVC inserted, and the name of the Healthcare Worker inserting the PIVC on the PIVC Insertion Record ([Appendix 3 and 4](#))

30. Sign for local anaesthetic if used.

18.2 Accessing the port

Procedure

1. Confirm patient identity and explain procedure to the patient
2. Perform hand hygiene
3. Apply non-sterile gloves
4. Using non-touch technique, disinfect access port by applying mechanical friction using a single-use 70% alcohol impregnated swab and allow to air dry
5. Flush PIVC with 0.9% normal saline for patency, then administer therapeutic substances
6. Flush PIVC with 0.9% normal saline post administration
7. Dispose of sharps and equipment
8. Remove gloves and perform hand hygiene.

18.3 Re-dressing the PIVC site

Requirements

- Trolley
- Dressing pack
- 2% Chlorhexidine in 70% alcohol swab stick or solution
- Sterile occlusive transparent dressing
- PPE as for standard precautions
- Waste disposal bag

Procedure

1. Confirm patient identity, check for allergies and explain procedure to the patient
2. Perform hand hygiene
3. Clean trolley with combined detergent / disinfectant wipe. Allow to dry
4. Gather equipment for the procedure
5. Perform hand hygiene and prepare field

6. Open dressing pack using corners, add sterile items and set out equipment using aseptic non-touch technique
7. Perform hand hygiene and apply non-sterile gloves and PPE as per standard precautions
8. Place waterproof/absorbent sheet under patient's arm
9. Remove old transparent dressing, take care not to remove PIVC
10. Advise patient to remain still to avoid accidental dislodgement (this may require extra nurse if patient is confused)
11. Dispose of used dressing in waste bag, remove non-sterile gloves
12. Perform hand hygiene and don non-sterile gloves
13. Cleanse skin liberally with 2% chlorhexidine in 70% alcohol swab stick or solution for 30 seconds. Allow to air dry. Secure PIVC with sterile occlusive transparent dressing over the insertion site using aseptic non-touch technique
14. Dispose of waste
15. Remove PPE. Perform hand hygiene
16. Clean trolley with combined detergent / disinfectant wipe and allow to dry
17. Document date and reason for dressing change in the patient's medical record.

18.4 Removal

Requirements

- Kidney dish
- Sterile gauze
- Sterile small dressing
- Disposal bag
- Personal Protective Equipment (PPE)

Procedure

1. Confirm patient identity and explain procedure to the patient
2. Perform hand hygiene
3. Disconnect intravenous tubing
4. Perform hand hygiene
5. Put on PPE as per standard precautions
6. Remove dressing from PIVC site and discard
7. Hold gauze over PIVC site and withdraw catheter. Apply pressure until bleeding stops. Apply sterile small dressing (keep intact for 24 hours)
8. If there are signs of infection, use a non-touch technique to avoid contamination of catheter and using sterile scissors cut off catheter tip and place into a sterile yellow top container and send to Microbiology
9. If the delivery system is fully assembled, discard into a disposal bag. If the giving set is separated from the infusion bag, remove the spike and place into a sharp's container
10. Dispose of waste.
11. Remove PPE and perform hand hygiene.

Compliance, monitoring and evaluation

All clinical staff who perform aseptic procedures have a responsibility to perform aseptic technique according to hospital policies, procedures and protocols.

HODs in each discipline are responsible for:

- ensuring all clinical staff who perform aseptic procedures are trained and performing within their scope of practice
- Ensuring relevant clinical staff have been assessed competent in aseptic technique.
- Compliance with Aseptic Technique e-Learning and Competency Assessment is regularly monitored and managed by HODs.

Compliance with this multidisciplinary Clinical Practice Guideline will be monitored through routine clinical incident review process.

Related Policies/Legislation/Standards

[MP 0038/16 Insertion and Management of Peripheral Intravenous Cannulae in Healthcare Facilities Policy](#)

Communicable Disease Control Directorate Guideline - [Insertion and Management of Peripheral Intravenous Cannulae in Western Australian Healthcare Facilities](#)

Related National Standards

[ACSQHC National Safety and Quality Health Service \(NSQHS\) Standards](#)

References

1. New South Wales Health, Clinical Excellence Commission. Ministry of Health. Intravascular access devices (IVAD) – infection prevention & control Sydney: NSW Health; 2019.
2. Australian Commission on Safety and Quality in Health Care. Management of Peripheral Intravenous Catheters Clinical Care Standard. Sydney: ACSQHC; 2021.
3. Australian Commission on Safety and Quality in Health Care. National standard for user-applied labelling of injectable medicines, fluids and lines. Sydney: ACSQHC; 2015.
4. [Australian and New Zealand College of Anaesthetists & Faculty of Pain Medicine. Have you had axillary lymph nodes removed during breast surgery 2025](#)
5. Australian Commission on Safety and Quality in Health Care. *Reprocessing requirements for ultrasound transducers used for percutaneous procedures* [Internet]. Sydney: ACSQHC; 2025 [cited 2026 Jan 27]. Available from: <https://www.safetyandquality.gov.au/publications-and-resources/resource-library/cleaning-and-disinfection-ultrasound-transducers>

Authorisation & Version Control

Policy Sponsor:	Area Director Clinical Services				
Policy Contact:	Clinical Nurse Consultant IV Access Service				
Version #:	2.0	Last Reviewed:	31/03/2026	Review Due:	31/03/2028
Reference #	CPG012	Consumer Engagement:	N/A	Aboriginal Health ISD #:	N/A
Approved by:	Area Director Clinical Services			Date:	31/03/2026
Endorsed by:	SCGOPHCG Clinical Practice Guideline Subcommittee			Date:	03/03/2026
Applicable Standards	NSQHS Standards: Std 1: Clinical Governance  Std 2: Partnering with Consumers  Std 3: Preventing and Controlling Healthcare Associated Infection 				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Appendix 1: PIVC device selection guide

This is a guide for PIVC device selection and should be used whenever practical. However clinical risks and patient characteristics may require a different size to be used.

PIVC SIZE	COLOUR	RATIONALE FOR USE
14G	ORANGE	Trauma patients Rapid, large-volume replacement
16G	GREY	High volume of fluids / Trauma patient's Major surgery Intra-partum or post-partum / GIT Bleeding Multiple line access Multiple blood transfers
18G	GREEN	Rapid administration of large volumes Blood products / Viscous fluid infusions Delivery of irritant medications Multiple line access Major surgery Coronary angiogram Imaging requiring power injection of CT contrast
20G	PINK	General use / IV maintenance IV antibiotics IV analgesia Imaging requiring power injection of CT contrast
22G	BLUE	Small or fragile veins Paediatric Most types of drug therapy - continuous intermittent or bolus Cytotoxic therapy
24G	YELLOW	Small veins For slow flow rates Neonatal Cancer services

(Insertion and Management of Peripheral Intravenous Cannulae in Western Australian Healthcare Facilities Policy MP 0038/16 v.1.1)

Selecting an appropriate site

- Consider the length of PIVC
- Start distally in the upper extremities
- Choose firm, round, elastic, well filled veins
- Assess the length of the vein
- Inspect and palpate for problems
- Look at or ask the patient for their previous history of cannulation (if possible).

Appendix 2: Peripheral Intravenous Assessment Score (PIVAS)

Peripheral Intravenous Assessment Score (PIVAS) Assessment criteria and actions required		
 Assess the need for the PIVC against the <u>DRIP</u> criteria below D Is there a risk of sudden Deterioration? R Is Rehydration needed, are IV fluids required? I Are IV medications required? P Are Procedures requiring IV access planned? If “No” to all the DRIP criteria, discuss alternative management options with the medical team before inserting a cannula. If a PIVC is insitu and if “No” to all DRIP criteria remove the PIVC.		
Assess the PIVC site each time it is accessed and document the PIVAS each shift and as per hospital Guidelines.		
<u>LOOK</u>	<u>LISTEN</u>	<u>FEEL</u>
Observe the PIVC site for erythema, swelling or exudate. Is the dressing clean, dry and intact?	Ask the patient or use visual clues. Is there pain or tenderness on infusion, palpitation or movement?	Palpate the site through the intact dressing. Is there any heat or vessel hardening?
Clinical Signs and Symptoms	PIVAS	Clinical Interventions
NO signs of phlebitis and/ or erythema, swelling, exudate, pain, tenderness and IV site appears healthy .	0	• Replace dressing if not clean, dry, or intact. • Remove if no longer required and continue PIVAS for 48 hours.
ANY evidence of ONE of the following signs / symptoms at or near the IV site - pain, tenderness or erythema.	1	• Remove cannula and continue PIVAS for 48 hours. • Document actions and discuss with medical officer. • Request re-siting if patient still meets DRIP criteria.
TWO of the following signs of Phlebitis are evident at or near the IV site: • Pain • Erythema • Swelling • Hardness • Purulent discharge • Palpable venous cord	2	• Remove cannula. • Document vital signs and discuss with medical officer. • Swab site if discharge present (consider blood cultures). • Re-site if patient still meets DRIP criteria. • Complete CIMS.
Fever $\geq 38^{\circ}$ C not explained by other causes.	 	Obtain 2 sets (4 bottles) of blood cultures from another peripheral vein. Send PIVC tip to pathology.

Appendix 3: Peripheral Intravenous Catheter (PIVC) Insertion and Observation Record



BM R320460

USE PATIENT LABEL WHEN AVAILABLE

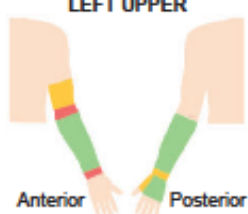
SIR CHARLES GAIRDNER OSBORNE PARK HOSPITAL CARE GROUP		URN: 	
PERIPHERAL INTRAVENOUS CATHETER (PIVC) INSERTION AND OBSERVATION RECORD		Surname: 	
		Forename: 	
		Gender: 	
		Ward: 	DOB:

Insertion Details PIVC no.

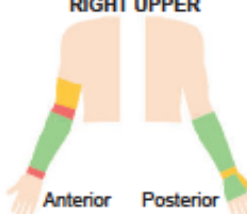
Date of insertion:	____/____/____	Avoid using veins in areas of flexion e.g. ACF or bony prominences.
Time of insertion:	____:____	
PIVC insertion site:	____	Insertion Rationale i.e. "DRIP" criteria: ☐ Deterioration ☐ Rehydration ☐ IV Medication ☐ Procedure ☐ Difficult IntraVenous Access (DIVA) i.e. no visible or palpable veins and/or limited (2 or less) sites for rotation. Escalate insertion to an experienced clinician. ☐ Referred to IV Access Service
PIVC gauge:	____ G	
Inserted by (print name):	____	
Department:	____	

Insertion Strategies:

☐ Local Anaesthetic
☐ Ultrasound

LEFT UPPER


Anterior Posterior

RIGHT UPPER


Anterior Posterior

PIVC location (please mark X)
 (Aim for "green" locations)
NB: Avoid areas of flexion
 ("red" & "yellow" locations)

 If PIVC not inserted in arm,
 please state location: _____

♦ If ambulance / emergency service insertion, remove when patient is stable or within 24 hrs. Re-site if rationale meets "DRIP" criteria.

Monitoring and Assessment

	Insertion Day			♦24 hours			48 hours			72 hours (Remove or re-site*)			Continue to monitor for 48 hours after removal, or until IV site healed					
DATE	/ /			/ /			/ /			/ /			/ /		/ /			
TIME	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND	AM	PM	ND
DRIP (letter) rationale																		
PIVAS 0																		
PIVAS 1																		
PIVAS ≥ 2																		
Initial																		

PIVC Removal

Removal reason:	Date: ____/____/____	Time: ____:____ hours
Removed by (print name):	Designation:	

☐ No longer required
☐ PIVAS 1
☐ PIVAS ≥2 (action as per recommendations over leaf)
☐ Tissued
☐ Patient removed / dislodged
☐ Other

PERIPHERAL INTRAVENOUS CATHETER
(PIVC) INSERTION AND OBSERVATION RECORD

 SCGH MR864.2 /
OPH MR120P(II)

HCHGA/PIVCR864P

M426
03/26



Appendix 4: Peripheral Intravenous Catheter (PIVC) Insertion and Observation Record – Difficult Intravenous Access (DIVA)



USE PATIENT LABEL WHEN AVAILABLE

SIR CHARLES GAIRDNER OSBORNE PARK HEALTH CARE GROUP

PERIPHERAL INTRAVENOUS CATHETER (PIVC) INSERTION AND OBSERVATION RECORD – DIFFICULT INTRAVENOUS ACCESS (DIVA)

URN: _____

Surname: _____

Forename: _____

Gender: _____

Ward: _____

DOB: _____

Insertion Details PIVC no _____

Date of insertion: ____/____/____

Time of insertion: _____

PIVC insertion site: _____

PIVC gauge: ____ G

Inserted by (print name): _____

Department: _____

Avoid using veins in areas of flexion e.g. ACF or bony prominences.

Insertion Rationale i.e. "DRIP" criteria:

☐ Deterioration ☐ Rehydration ☐ IV Medication ☐ Procedure

DIVA i.e. no visible or palpable veins and/or limited (2 or less) sites for rotation. Escalate insertion to an experienced clinician.

☐ Referred to IV Access Service

Insertion Strategies:

☐ Local Anaesthetic

☐ Ultrasound

LEFT UPPER

Anterior Posterior

RIGHT UPPER

Anterior Posterior

PIVC location (please mark X) (Aim for "green" locations)

NB: Avoid areas of flexion ("red" & "yellow" locations)

If PIVC not inserted in arm, please state location: _____

♦If ambulance / emergency service insertion, remove when patient is stable or within 24 hrs. Re-site if rationale meets "DRIP" criteria.

Monitoring and Assessment

NB: *Exception to 72-hour removal / re-siting applies to DIVA patients only, where extended dwell time is clinically indicated, and the PIVAS and clinical assessment supports retention of PIVC.

	Insertion Day	♦24 hours			48 hours			72 hours (Remove or re-site*)			DIVA only*					
											96 hours			120 Hours		
DATE	____/____/____	____/____/____			____/____/____			____/____/____			____/____/____			____/____/____		
TIME	AM PM ND	AM PM ND	AM PM ND	AM PM ND	AM PM ND	AM PM ND	AM PM ND	AM PM ND	AM PM ND	AM PM ND	AM PM ND	AM PM ND	AM PM ND	AM PM ND		
DRIP (letter) rationale																
PIVAS 0																
PIVAS 1																
PIVAS ≥ 2																
Initial																
Continue to monitor for 48 hours after removal, or until IV site healed																
DATE	____/____/____															
TIME																
D.R.I.P (letter) rationale to remain	Not applicable															
PIVAS 0																
PIVAS 1																
PIVAS ≥ 2																
Initial																
PIVC Removal																
Removal reason:								Date: ____/____/____	Time: ____:____ hours							
Removed by (print name):								Designation:								
☐ No longer required ☐ PIVAS 1 ☐ PIVAS ≥2 (action as per recommendations over leaf) ☐ Tissued ☐ Patient removed / dislodged ☐ Other																

M821
03/26

MR SCGH MR863.3 / PERIPHERAL INTRAVENOUS CATHETER (PIVC) INSERTION AND OBSERVATION RECORD – DIFFICULT INTRAVENOUS ACCESS (DIVA)
OPH MR120.3